



THE DEMENTIA HOUSE CALL

A STAGE-BY-STAGE GUIDE FOR FAMILIES

The Late-Stage Guide

Memory care, comfort, and the final months



Plain, practical guidance from a doctor who makes dementia house calls

Educational information for caregivers — not medical advice.

HOW TO USE THIS GUIDE

Read only the page you need

This is a broad guide, meant to be read a page at a time. You do not need to read it front to back — start with whatever you're facing now, and come back to the rest when you have room.

THE ONE RULE THAT RUNS THROUGH EVERYTHING

A **sudden** change — new confusion, agitation, or sleepiness over hours or a day or two, often with a fever, pain, a change in urine, or after a new medication — is **medical until proven otherwise**. That is usually delirium, it is frequently reversible, and it needs a phone call, not a behaviour strategy. When something changes fast, think medical first.

A NOTE ON THE FOOTNOTES

A few pages point to products or services that genuinely help at this stage. Those are honest suggestions, not prescriptions — use only what helps, always with your person's knowledge. Some are affiliate links, which means The Dementia House Call may earn a small commission at no extra cost to you; it never changes the guidance, and we never steer a medical decision toward a purchase.

WHERE YOU ARE NOW

The last chapter

By the late stage, the disease has taken most of what it will take. Words narrow to a few, then to none. Walking gives way to sitting, sitting to lying; swallowing becomes hard; sleep fills more of the day. Your person needs help with everything now.

And yet — this is the part families are rarely told, so hear it plainly — **they still feel**. Touch still reaches them. A familiar voice still soothes. Music they've loved their whole lives can still light something in a face that has gone quiet. The person is still in there, reachable not through facts or memory but through **comfort, presence, and the senses**.

Your job changes here. You are no longer fighting the daily battles of the middle stage. You are now the **guardian of their comfort and dignity** — the one who makes sure they are warm, out of pain, clean, and not alone. It is quieter work, and in its way harder, because so much of it is simply *being there*. This guide is about doing it well, and about looking after yourself while you do.

THE MOVE TO MEMORY CARE

When home isn't enough anymore

For most families, the late stage is when caring at home stops being possible or safe — round-the-clock needs, lifting, incontinence, the sheer relentlessness. **Moving your person into memory care is not a failure and not abandonment**. It is often the most loving thing left to do: it gets them safe, skilled, 24-hour care, and it lets you go back to being their *son, daughter, husband, wife* instead of their exhausted nurse.

CHOOSING A HOME — WHAT TO ACTUALLY LOOK FOR

- **Secure memory-care unit** (not just assisted living): staff trained in dementia, a locked/monitored floor, a calm layout.
- **Staffing ratios and turnover** — ask directly; watch how staff speak to residents, not just about them.
- **How they handle the hard things:** behaviours, incontinence, end-of-life, hospital transfers. Do they have a plan for comfort-focused care?
- **Visit unannounced**, at a mealtime and in the late afternoon (sundowning hours). Trust what you see and smell.

You don't have to find it alone. Free senior-living placement advisors know the local homes, the openings, and the costs, and will shortlist options that fit — at no charge to you.¹ It's the fastest way to answer the question that flattens most families at this point: *"who do I even call?"*

AFFILIATE NOTE

¹ Free senior-living placement services — e.g. [A Place for Mom](#) or [Caring.com](#). They're free to families; use them as a starting shortlist, then trust your own eyes on the visit.

BEING THEIR ADVOCATE

Your job when you're no longer the hands

Moving them into a home doesn't end your role — it changes it. You become their **voice and their watchdog**.

- **Make them a person to the staff.** Bring the life-story book (if you made one earlier, this is its moment): who they were, what they love, what calms them, what they can't stand. Staff care better for a person they know.
- **Build the relationship, not the complaint file.** Warm, specific, regular contact with the team gets your person better care than confrontation. But **document** what matters, and raise concerns early and clearly.
- **Watch for the things they can't report:** weight loss, new bruises, pressure areas, a change in mood or alertness, untreated pain.
- **Know their wishes are followed** — the goals-of-care and comfort decisions you made together should be in their chart and honoured.

WHAT "COMFORT CARE" MEANS

Changing the goal

Somewhere in the late stage, the goal of care quietly changes — from *adding time* to *protecting comfort*. This is **comfort-focused (palliative) care**, and choosing it is not giving up. It is deciding that how they feel matters more than numbers on a chart.

In practice it means asking, of every test, hospital trip, and treatment: **will this make them more comfortable, or just prolong this?** Often the kindest answers are the gentler ones — treating a chest infection with care at the home rather than a frightening hospital transfer; skipping the routine bloodwork; focusing on pain, calm, and presence. These are conversations to have *with the medical team, ahead of a crisis*, so decisions aren't made in a panic at 2 a.m.

COMFORT IN THE BODY

Skin, mouth, and pain

As a person stops moving and eating, the body needs different, gentle attention. This is where good late-stage care is quietly won.

- **Skin & pressure areas.** A person who can't shift themselves is at real risk of pressure sores. **Prevention is everything:** regular repositioning, pressure-relieving cushions and mattresses, and a daily look at the pressure points (heels, hips, tailbone, elbows).² Catch redness early.
- **Mouth care.** A dry or sore mouth is a common, hidden misery when someone eats and drinks little. Gentle, frequent mouth care keeps them comfortable.
- **Pain you can't see.** They may not be able to tell you. **Read the body:** a furrowed brow, grimacing, guarding, tension, moaning, agitation at being moved. Assume pain is possible, and insist it be treated — untreated pain is often the real cause of "difficult" behaviour at this stage.

² A pressure sore is a wound — it needs a nurse, not a shopping cart. This section is about **prevention** (repositioning, skin checks, the right cushion); anything that looks like a breaking-down area needs assessing, not managing. If you're unsure, assume it does and make the call.

EATING & SWALLOWING AT THE END

When they can't eat well anymore

In late dementia, the body forgets how to swallow safely. Food and drink go down the wrong way; weight falls despite everyone's best efforts. This is one of the most painful parts for families, because feeding is love.

Two things worth knowing, from the evidence:

- **Careful hand-feeding, for pleasure and comfort, is the kindest approach** — small tastes of favourite things, at their pace, for as long as they enjoy it.
- **Feeding tubes do not help in advanced dementia.** They don't prolong life, don't prevent pneumonia, don't heal pressure sores, and can cause distress and restraint. Major geriatric bodies recommend *against* them, in favour of comfort feeding. (This is covered in full in the site's eating letter — the most important paragraph on the site.)

Letting go of "they must eat more" in favour of "they should be comfortable" is not starving them. It's meeting the body where it is, with kindness.

HOSPICE

The word that sounds like the end, and isn't

Hospice is not a place you send someone to die in a week. It's a service — often brought to the care home or your home — focused entirely on comfort: expert pain and symptom control, equipment, nursing, and support for the whole family. Many people are eligible far earlier than families realise, and those who start hospice often do *better*, not worse, because someone is finally managing comfort as the whole job.

Ask about it early. If you find yourself wondering "is it time?", that's usually the sign to have the conversation. You lose nothing by asking, and you may gain months of better comfort — for them, and real support for you.

VISITING WHEN THEY DON'T KNOW YOU

Presence over recognition

One of the deepest griefs of the late stage is the day they don't know your name. Here is what helps: **they may lose the label and still keep the feeling.** They may not know *that* you're their daughter, but they can still feel that you are safe, warm, and theirs.

- **Lead with the senses.** A hand held, a gentle voice, a favourite song, a familiar smell, a soft blanket. Music especially reaches people when words no longer can.
- **Drop the quiz.** No "do you know who I am?" It only surfaces the loss for both of you. Just arrive, warmly: *"Hi Mum, it's so good to see you."*
- **Match their world.** Sit close, go slow, follow their pace. A calm ten minutes of presence is worth more than an hour of trying to reach the person they were.
- **Let it be enough.** You being there lands, even when nothing comes back. Especially then.

THE LAST MONTHS

Being there at the end

As the very end approaches, the signs are usually gentle: more sleep, less interest in food and drink, withdrawing, changes in breathing. This is the body slowing to a stop, and — well-supported — it is usually peaceful.

- **Keep them comfortable:** pain and symptoms managed, mouth moist, skin cared for, warm, clean, not alone.
- **Keep the senses close:** your voice, soft music, a held hand. Hearing is thought to remain until very near the end — so keep talking to them, keep saying what you need to say.
- **Lean on hospice** for what to expect and how to respond, so nothing frightens you.
- **You don't have to be strong. You have to be there** — and even that, in your own way, is allowed to have limits.

GRIEF

The grief that started long ago

You have been grieving this person for years already — every ability lost, every "last time" you didn't know was the last. That **anticipatory grief** is real, and it means the grief after death is often tangled: sorrow and relief at once, and then guilt for the relief. Please hear this clearly: **relief is not a failure of love. It is the natural response to the end of long suffering — theirs and yours.**

- Be gentle with yourself; grief after a long caregiving road has its own strange shape.
- Reach for support — a group of people who've walked this exact road, or a counsellor.³
- Let the good memories back in, in time. The disease had the last chapter, but it doesn't get the whole book.

³ Caregiver and grief support — the site's "For you" letter, a local hospice's bereavement service, or a structured support programme. You do not have to carry the after alone either.



THE DEMENTIA HOUSE CALL

You don't have to figure this out alone

The Dementia House Call sends free, plain-language letters for families — the same things a good house call would tell you. Join at **thedementiahousecall.com**, and look for the rest of the stage-by-stage series and the Care Binder.

The series: The Early-Stage Guide · The Middle-Stage Survival Guide · The Late-Stage Guide · and the Care Binder, the filing cabinet they all point back to.

This guide is general education for families and is not medical advice; it does not create a doctor–patient relationship and does not replace your own physician. Anything that looks like a sudden change, an infection, or an injury needs assessing, not managing — if you're unsure, assume it does and make the call. Some links in this guide are affiliate links; The Dementia House Call may earn a small commission at no cost to you, and it never changes the guidance. © 2026 The Dementia House Call.